

Report to	IMEDICAL, 1 Union st , Pyrmont, NSW, 2009	Patient	SCHLEGER, SHANE IMEDICAL C/O IMEDICAL 1 UNION ST PYRMONT NSW 2009		
		Phone	0415994541		
		D.O.B	01/01/1985	Age	40 years
				Sex	M
Ref. by/copy to	IMEDICAL,	Collect date	01/05/2025	Lab ref	25-12980832
		Collect time	09:30 AM	Your ref	
		Reported	02/05/2025		03:30 PM
Tests requested	GH, SCP, COR, AND, HOR, SOM, TFT, PRL, FE, MBA, LIP, GLU, FBE				
Clinical notes					

SERUM GROWTH HORMONE

Growth Hormone < 1.0 mIU/L (< 3.1)

In children, Growth Hormone should be > 20 mIU/L following any appropriate stimulus (e.g. sleep, exercise, hypoglycaemia, arginine). Random values are usually low. Growth Hormone is not recommended to diagnose Growth Hormone excess or deficiency. IGF-1 is the preferred marker, especially for the investigation of Growth Hormone excess.

SURGERY
USE

Normal

No Action/File

Patient
Notified

Make
Appoint.

Further Tests

Notes
Required

Speak
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On Correct
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SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)

Request Number 12980832
Date Collected 1 May 25
Time Collected 09:30
hsCRP (< 4.91) mg/L 0.89

The CDC / AHA recommend the following hsCRP cut-off points (tertiles) for cardiovascular disease risk assessment:

hsCRP level (mg/L)	Relative Risk
<1.0	Low
1.0 - 3.0	Average
>3.0	High

The average of two CRP tests, ideally taken two weeks apart, produces a more stable estimate of this marker.

A CRP greater than 10mg/L should prompt a search for a source of infection or inflammation.

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SERUM CORTISOL

Time 09:30
Cortisol 534 nmol/L

AM Reference Interval 120-620 nmol/L

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SERUM ANDROGENS

Total Testosterone (Siemens)	16.9	nmol/L	(8.3-29)
Sex Hormone Binding Globulin	42	nmol/L	(11-71)
DHEAS	7.6	umol/L	(3.0-10)
Calculated Free Testosterone	300.4	pmol/L	(255.0-725.0)

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SERUM HORMONE PROFILE

Specimen Type: Serum								
Request	Date	FSH	LH	PROG	E2(ATEL)	E2(BECK)	LH/FSH	
Number	Collected	IU/L	IU/L	nmol/L	pmol/L	pmol/L	Ratio	
12980832	1 May 25	11	3.5	2	140			
Reference Ranges		FSH	LH	PROG	E2(ATEL)			
Males < 60 yrs		<7	<7	0.9-3.9	<150			
> 60 yrs		<14	<11	0.9-3.9	<150			

PLEASE NOTE:
'E2 (ATEL)' - Oestradiol by Siemens Atellica assay
'E2 (BECK)' - Oestradiol by Beckman Access assay

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INSULIN-LIKE GROWTH FACTOR-1 (IGF-1)

Specimen Type: Serum

IGF-1 (Somatomedin C) 19 nmol/L (13-36)

Insulin-like growth factor 1 (IGF1) and insulin-like growth factor-binding protein 3 (IGFBP3) measurements can be used to assess growth hormone (GH) excess or deficiency. For all applications, IGF1 measurement has generally been shown to have superior diagnostic accuracy. In particular, for the diagnosis and follow-up of acromegaly and gigantism, IGFBP3 measurement adds little extra diagnostic information.

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THYROID PROFILE

Specimen Type: Serum			
TSH	2.0	mIU/L	(0.5-4.0)
FT4	14	pmol/L	(10-20)
FT3	5.8	pmol/L	(3.5-6.5)

Result(s) consistent with euthyroidism.

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PROLACTIN

Specimen Type: Serum

Request Number	Date Collected Ref Range	Prolactin mIU/L (40-450)
12980832	01/05/25	324

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IRON STUDIES

Specimen Type: Serum			
Serum Iron	10	umol/L	(10-30)
Transferrin	25	umol/L	(27-46)
Transferrin Saturation	20	%	(13-45)
Serum Ferritin	142	ug/L	(30-300)

Transferrin may be decreased by inflammation (acute or chronic), or protein deficiency or loss. The ferritin concentration excludes iron deficiency.

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SERUM CHEMISTRY

Specimen Type: Serum

Haemolysis	Nil		
Icterus	Nil		
Lipaemia	Nil		
Sodium	141	mmol/L	(135-145)
Potassium	4.2	mmol/L	(3.6-5.4)
Chloride	104	mmol/L	(95-110)
Bicarbonate	26	mmol/L	(22-32)
Anion Gap	15	mmol/L	(10-20)
Urea	7.5	mmol/L	(2.5-7.5)
Creatinine	110	umol/L	(60-110)
eGFR	72		mL/min/1.73sqM
Urate	0.36	mmol/L	(0.20-0.42)
Bilirubin	8	umol/L	(< 20)
AST	26	U/L	(< 40)
ALT	28	U/L	(< 40)
GGT	14	U/L	(< 50)
Alkaline Phosphatase	65	U/L	(35-110)
Protein	68	g/L	(60-82)
Albumin	44	g/L	(38-50)
Globulin	24	g/L	(20-38)
Calcium	2.36	mmol/L	(2.10-2.60)
Corrected Calcium	2.34	mmol/L	(2.10-2.60)
Phosphate	1.24	mmol/L	(0.75-1.50)
Magnesium	0.84	mmol/L	(0.70-1.10)

eGFR values between 60 and 89 mL/min/1.73m² should be interpreted with caution. These results are only consistent with CKD in the presence of other evidence such as microalbuminuria, proteinuria or haematuria.
Ref:Lamb EJ etal in Ann Clin Biochem 2005; 42:321-345.

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LIPID STUDIES

Specimen Type: Serum

Reference intervals are included for reference only, and interpretation / treatment goals should be guided by patient-specific cardiovascular risk assessment (see Australian Cardiovascular Risk Charts. Alternatively, the web-site www.cvdcheck.org.au can be accessed in order to complete a risk assessment for individual patients.)

Haemolysis	Nil
Icterus	Nil
Lipaemia	Nil

Fasting status	Fasting		
Total Cholesterol	3.7	mmol/L	(3.9-5.2)
Triglycerides	1.1	mmol/L	(0.5-1.7)
HDL Cholesterol	1.0	mmol/L	(1.0-2.0)
LDL Cholesterol	2.2	mmol/L	(1.5-3.4)
Non-HDL Cholesterol	2.7	mmol/L	(< 3.4)
Cholesterol/HDL-C Ratio	3.7		(< 5.0)

NVDPA TARGET LIPID RANGES (MMOL/L) FOR PATIENTS AT HIGH / MODERATE RISK OF CARDIOVASCULAR DISEASE:

TOTAL CHOLESTEROL	<4.0
TRIGS (FASTING)	<2.0
HDL -C	>= 1.0
LDL -C	<2.0
NON HDL -C	<2.5

LDL-C exceeds target for higher risk patients and may be excessive in some individuals.

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SERUM/PLASMA GLUCOSE

Fasting status	Fasting		
Serum	4.9	mmol/L	(3.4-5.4)
Normal glucose concentration.			

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HAEMATOLOGY

Date Collected 01 May 25
Time Collected 09:30
Specimen Type: EDTA

Hb	154	g/L	(130-180)	WBC	6.1	x10 ⁹	/L (4.0-11.0)
RCC	5.2	x10 ¹²	/L (4.5-6.5)	Neut	2.7	x10 ⁹	/L (2.0-7.5)
Hct	0.46		(0.40-0.54)	Lymp	2.7	x10 ⁹	/L (1.0-4.0)
MCV	87	fL	(79-99)	Mono	0.4	x10 ⁹	/L (0.2-1.0)
MCH	29	pg	(27-34)	Eos	0.3	x10 ⁹	/L (< 0.7)
MCHC	338	g/L	(320-360)	Baso	0.1	x10 ⁹	/L (< 0.2)
RDW	12.2	%	(10.0-17.0)				
Plat	270	x10 ⁹	/L (150-400)				

HAEMATOLOGY: FBC parameters are within reference range.